ROYAL FITNESS CLUB

RFC PREMIUM GUIDE #07

SARMS COMPLETE SCIENTIFIC HANDBOOK

Evidence-Based Guide to Selective Androgen Receptor Modulators

5 Chapters · Scientific Reference · Harm Reduction · Blood Work Protocols

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DISCLAIMER

This guide is for educational and harm-reduction purposes only. It does not constitute medical advice. Consult a qualified healthcare professional before starting any supplementation or pharmaceutical protocol. Individual results vary.

CHAPTER 1 — WHAT ARE SARMs?

"SARMs are tissue-selective androgens — the goal of decades of pharmaceutical research to separate the muscle-building effects of testosterone from its other effects."

Selective Androgen Receptor Modulators (SARMs) are a class of therapeutic compounds that bind to androgen receptors with tissue selectivity — activating them in muscle and bone while minimising activation in the prostate, scalp, and other androgen-sensitive tissues. This selectivity profile makes them theoretically safer than anabolic steroids for performance enhancement, though long-term human safety data remains limited.

SARMs Category	Selectivity	Research Stage	Examples
Steroidal SARMs	Moderate	Historical/abandoned	Early DHT derivatives
Non-steroidal SARMs	High	Phase I–III trials	Ostarine, LGD-4033, RAD-140
SARM-like (SPPARM)	Very high	Phase I–II	Enobosarm, MK-677
Investigational SARMs	Experimental	Pre-clinical	YK-11, S-23, ACP-105

CHAPTER 2 — INDIVIDUAL COMPOUND PROFILES

SARM	Half-Life	Common Dose	Primary Effect	Suppression Level
Ostarine (MK-2866)	24 hrs	15–25mg/day	Lean muscle, joint healing	Low–Moderate
LGD-4033 (Ligandrol)	24–36 hrs	5–10mg/day	Significant lean mass	Moderate–High
RAD-140 (Testolone)	16–20 hrs	10–20mg/day	Hardening, strength	Moderate–High
Cardarine (GW-501516)	16–24 hrs	10–20mg/day	Endurance, fat oxidation	None (PPAR-δ)
Andarine (S4)	4 hrs	25–50mg/day	Vascularity, hardening	Moderate
YK-11	6–10 hrs	5–10mg/day	Myostatin inhibition, mass	High
MK-677 (Ibutamoren)	24 hrs	12.5–25mg/day	GH secretagogue, sleep	None (not SARM)

■ ***Note: Cardarine (GW-501516) is not technically a SARM — it is a PPAR-δ agonist. It was discontinued by GSK due to cancer findings in animal models at high doses. Use with extreme caution.***

CHAPTER 3 — BULKING vs CUTTING PROTOCOLS

Lean Bulking Stack

Compound	Dose	Duration	Purpose
LGD-4033	5–10mg/day	8–12 weeks	Primary mass builder
MK-677	12.5–25mg/day	16+ weeks	GH secretagogue, sleep, recovery
Cardarine (optional)	10mg/day	8–12 weeks	Endurance, prevents fat gain
Ostarine (bridge)	12.5mg/day during PCT	4 weeks	Muscle preservation during PCT

Cutting Stack

Compound	Dose	Duration	Purpose
Ostarine	20–25mg/day	10–12 weeks	Muscle preservation in deficit
Cardarine	20mg/day	10–12 weeks	AMPK activation, fat oxidation
Andarine S4	25mg/day	8–10 weeks	Hardening, vascularity
RAD-140 (optional)	10mg/day (wk 5-10)	6 weeks	Strength + conditioning

CHAPTER 4 — SARMs vs STEROIDS: RISK-BENEFIT

Criterion	SARMs	Anabolic Steroids
Muscle gain (12 weeks)	3–6kg lean mass	6–15kg (compound dependent)
Liver toxicity	Low–Moderate (oral)	Moderate–High (17-aa orals)
Cardiovascular risk	Low–Moderate	Moderate–High
HPTA suppression	Low–Moderate	Significant–Complete
Estrogenic effects	None (most SARMs)	Present (aromatising compounds)
Hair loss risk	Low (most SARMs)	High (DHT derivatives)
Virilisation (women)	Minimal (low doses)	Significant
Legal status	Research chemical	Schedule III (US), various

CHAPTER 5 — SUPPRESSION, PCT & BLOOD WORK

"Suppression is a spectrum, not a binary. Even mild SARMs cause measurable LH and FSH reduction — this matters."

SARMs suppress the Hypothalamic-Pituitary-Testicular Axis (HPTA) to varying degrees. Unlike steroids, complete suppression is rare, but partial suppression is nearly universal with even "mild" SARMs like Ostarine at doses above 15mg/day for 8+ weeks. PCT helps restore endogenous testosterone production faster.

SARM Cycle	Suppression Level	PCT Required?	PCT Protocol
Ostarine < 15mg, < 6 wks	Minimal	Optional (mini PCT)	Nolvadex 20mg/day × 4 wks
Ostarine full dose, 10–12 wks	Low–Moderate	Yes	Nolvadex 40/40/20/20
LGD-4033 standard	Moderate	Yes	Clomid 50/25 + Nolvadex 40/20
RAD-140 standard	Moderate–High	Full PCT	Clomid 50/50/25/25 + Nolvadex 40/40/20/20
YK-11 standard	High	Full PCT	Same as steroid cycle PCT

- Pre-cycle bloods: testosterone total + free, LH, FSH, SHBG, CBC, CMP, lipids
- Mid-cycle check (Week 6): testosterone, LH — confirms suppression degree
- Post-PCT bloods: 4 weeks after PCT completion — testosterone should be >80% of baseline
- If testosterone does not recover within 16 weeks post-cycle, see an endocrinologist